Getting Mum **to her appointment.**

One place to arrange an older relative's medical appointment and the ride to it — and live proof, minute by minute, that it is actually happening.

15 SEPTEMBER 2026 · SUMMARY OF THE FULL PROJECT OVERVIEW

The problem

An adult child, often in another city, gets a call: Mum has a cardiology follow-up on Thursday at 10:40, and she no longer drives. What follows is not a clinical problem but a logistics one that no single system owns — calls to the clinic, to a transport company, explaining the walker and the wheelchair, booking a return leg without knowing when the appointment will end, and then waiting. Every party holds one fragment: the clinic knows the slot but not whether the patient is on the way; the operator knows the driver but not when the appointment ends; the family knows only that an appointment exists.

> **WHAT ACTUALLY DRIVES PEOPLE AWAY**
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> Not a missed appointment — **the two hours of silence in the middle of a workday**, between "the car should have arrived by now" and someone finally picking up a phone. Removing that interval is the whole product.

What CareBridge does

It puts the appointment, the ride, the live position of the car and the payment in one place, and keeps everyone who should know informed without anybody having to ask. A family member opens one app and can answer three questions without interacting with it: **What is next? Is it on track? What do I do if it is not?**

The care circle	A sibling, neighbour or paid carer is invited in and sees the same schedule and the same alerts — not just whoever made the booking. Invitations are single-use, expiring and tied to one verified address.
Appointments	Recorded with clinic, time and expected duration, with reminders set against the clinic's own local time. Recorded in CareBridge — not booked into a clinic's own system.
Transport	A round trip is two separate journeys, because nobody knows when an appointment will end. Each is assigned, tracked and priced on its own, with an itemised estimate shown before confirming.
Dispatch	A dispatcher works a queue ordered by when each car is actually needed. Access needs are enforced as hard constraints: a wheelchair journey cannot be given to a vehicle that cannot carry one.
Live tracking	The family watches the car with an arrival time counting down, and is notified at each step. **If the position goes stale the app says so**, and past two minutes it stops showing a position at all.
Money and record	Quoted up front, authorised at assignment, charged on completion — then a receipt and a complete timeline, which is also what both sides reach for when they disagree.

The live map is what turns an administrative product into an emotional one. It replaces a phone call, it bounds anxiety ("six minutes away" is a tolerable wait; "no update since 09:12" is not), it produces evidence for disputes, and it is the way in to the transport operator — operators adopt CareBridge because *their* customers stop calling them, not because they wanted new software.

4	2	~85%	0
Users served — family, patient, driver, dispatcher	Paying sides — households and operators	Of the planned product built and demonstrable	Real journeys carried — the pilot has not begun

Where it stands today

A complete journey runs end to end right now — booking, dispatching, assigning a driver, watching the car arrive, the return leg, the receipt — with everything stored permanently and nothing reset between sessions. Four surfaces exist and are usable: the family app (with a simplified mode for the patient, who may never install anything), the driver app, the dispatcher's console and a back office.

WORKING	The full family journey · dispatch and the driver lifecycle · the driver app, including a queue of taps that survives a tunnel · both subscriptions · account security in full · fares priced from editable rules
STAND-IN	Maps, card payments, email and push notifications all work but point at test services rather than live vendor accounts · assignment is a human dispatcher, not automatic matching · **no real car has driven a real route yet**
NOT BUILT	**Account deletion** — designed, not built, and the single item blocking both app stores · screens for the caregiver marketplace and clinic portal (built behind the scenes, deliberately switched off) · a live hosted environment · a rehearsed restore from backup · the pilot paperwork

How it makes money

Two payers, one mechanism. The original model had families paying a flat fee plus a percentage of each fare — which made revenue rise exactly when the operator's own margin was thinnest, and gave them a reason to route busy days around us. The operator was also the party whose adoption decided whether the product existed at all. So both sides now subscribe.

	THE FAMILY	THE TRANSPORT OPERATOR
What they buy	Coordination — booking, tracking, reminders, the care circle	Operations — the dispatch console, the driver app, assignment
Priced by	Household	**Drivers on the road**
Also pays	The fare for each journey	—

Drivers on the road is the only number an operator already tracks and the only one that moves with the value they get. The ladder is graduated, so a bill never *falls* when they hire. When an operator is on a plan the whole fare passes through to them — charging a percentage as well would be charging twice for one relationship, which they find in a spreadsheet six months in and never forgive. The family pays the same total either way, deliberately. Costs are light: no vehicles, no drivers employed, and the only line that scales with use is map and routing, which is throttled, cached and alarmed.

The market

Three kinds of product sit near this problem and each solves a fragment. Non-emergency transport brokers move people and tell families nothing. Consumer ride-hailing tracks beautifully but cannot handle a wheelchair, a scheduled pickup or a passenger who needs an arm to lean on. Caregiver marketplaces staff visits and ignore transport entirely. The defensible position is the **join** between them — and starting at any of those other layers makes coordination a retrofit.

> **THE CONSTRAINT ACCEPTED KNOWINGLY — AND WHERE I WOULD VALUE YOUR VIEW**
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> CareBridge takes payment by card only; it does not bill insurance, Medicaid or Medicare. That narrows the addressable market substantially, and was accepted on the reasoning that building payer integration first would mean building the hardest part for a product nobody has yet said they want. My assumption is that a meaningful segment of families already pays privately — because a parent is not eligible, because the eligible option is unreliable, or because they would rather not deal with the paperwork. That assumption is the one the whole commercial plan rests on, and your experience is worth considerably more than my analysis of it.

Before a pilot can run

Most of what remains is arrangements rather than construction, in this order:

- **A hosted environment** — a cloud account. Until one exists there is nothing for a phone to talk to.
- **Four vendor accounts** — email, maps, push, payments. Each is written and waiting. Without email nobody can sign up at all.
- **The field test** — one car, one route, half a day. It settles the battery and coverage claims, which are currently designed for rather than proven.
- **Account deletion** — the one remaining build item blocking both stores.
- **Consent and a privacy policy** — the system holds sensitive detail about people who are usually not the account holder.
- **A rehearsed restore**, and a decision about who answers the phone at eight on the first Monday.

Two items are pure waiting and can start immediately: the app store's organisational enrolment, and the email provider's release from its restricted mode. Both take days to weeks and block everything downstream.

How we will know it is working

Every measure carries a stop condition — a number that, if hit, means something is wrong with the idea rather than the execution. The one that matters is **calls to dispatch per completed journey**: baseline measured with the pilot operator beforehand, target a 50% reduction within two months, and a stop condition of no measurable reduction after 100 tracked journeys. Alongside it: families active weekly above 70%, journeys tracked end to end above 90%, trial conversion above 25%, and churn under 5%. Four things are treated as incidents with no acceptable number, among them a location visible to anyone without a grant, and a notification containing a patient name, clinic or time.

> **ON THE CLINICAL BOUNDARY — MY REASONING, OFFERED FOR YOUR CORRECTION**
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> CareBridge holds no diagnoses, no medications, no clinical notes and no lab results, offers no advice, and is not an emergency service. My reasoning was that coordination data is sensitive but bounded, and that the moment the system stores a diagnosis it becomes something quite different — a step better taken deliberately and late than by accident. My second claim is the one I would most like you to test: that nothing in arranging transport actually requires knowing *why* someone is seeing a specialist. If there is a category of patient for whom that is wrong, I would rather hear it now than discover it in a pilot.

Three questions I would put to you

- Is the clinical boundary in the right place — is there genuinely nothing about arranging transport that requires clinical detail?
- How large, in your experience, is the segment of families already paying privately for medical transport?
- Would a clinic front desk actually use a portal showing expected arrival times, or is that one more screen nobody logs into? It is built and switched off, so the answer costs nothing to act on either way.

IN ONE SENTENCE

CareBridge works today, end to end, and the gap between here and a real pilot is mostly accounts, arrangements and one afternoon in a car — not more building. The main open questions are commercial rather than technical.

CareBridge coordinates medical appointments and transport. It holds no medical records, offers no medical advice, and is not an emergency service. Nothing here should be read as a claim of compliance with any particular regulatory regime; the system has been designed with those requirements in view, but compliance itself depends on legal determinations, executed agreements and operational practices that sit outside the product.